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A. Purpose: To provide guideline instruction for surgical care services with the aim that

a. Need and expectations of the patients are established.
b. Patient satisfaction is enhanced on a continuous basis.

B. Scope: It covers all patients undergoing any form of surgical treatment in the hospital.

C. Responsibility Person: Senior consultant- Surgery

D. Departmental Hierarchy:

Senior Consultant – Surgery
↓
Consultant - Surgery
↓
Staff Nurse
↓
Housekeeping Staff.

E. Policy:

1. Qualification of staff:

a. Surgical care is provided by post graduate qualified and experienced surgeons.
b. Anaesthesia is provided by post graduate qualified and experienced anaesthesia.
c. Nursing care in the OT is provided by qualified and registered nursing staff.

Orthopedics and trauma:

The Orthopedic department at Hope Hospital takes care of all kinds of Orthopaedic & trauma situations. The 24 hour Accident and Emergency (A & E) department receives and takes care of all major and minor trauma and other Orthopedic emergencies. Our organisation treats polytraumatized patients. Orthopedic surgery plays a key role in the treatment of the polytraumatized patient. The orthopedic surgeon is a member and head of team consisting of many specialists, including emergency medicine physician, general surgeon, neurosurgeon, plastic surgeon, vascular surgeon and other surgical subspecialists like faciomaxillary surgeon. Each person on that team is familiar with the basics of trauma resuscitations.

Neurosurgeries are not routinely performed in our organisation. Emergency decompressive craniotomies and similar surgeries to save a life of a dying polytrauma patient may be performed and

later transferred to superspeciality center specialising in Neurosciences. Neurosurgeons are the only ones trained to perform procedures inside the lining of the spinal canal. There are specific medical conditions that require this expertise, including spinal cord tumours, cysts, Chiari malformation, arteriovenous malformation and spina bifida. Spinal surgeons are drawn from two specialty areas – neurosurgery and orthopaedics. The margins between them have now blurred. But those from orthopaedics in our organisation deal with deformity in the spine, such as scoliosis and disorders of the thoracic and lumbosacral spine. Those from a neurosurgical background deal with more specialised surgery involving the brain and cervical spine. Neurosurgery facilities in Hope Hospital include Carl Zeiss Microscopy and neurosurgery instruments.

The Orthopedic department is managed by full time consultants each of them having many years of experience in various fields of Orthopedic surgery.

Orthopedic services include:

Medical and surgical treatment of various Orthopedic disorders
Joint replacements surgeries of the hip, knee and shoulder joints
Arthroscopic surgeries of various joints
Treatment of ligament injuries and sports injuries
Limb reconstruction and Micro-Vascular surgeries & hand surgery Ilizarov ring fixator treatment for various Orthopedic and trauma situations Correction of deformities of upper and lower limbs Surgery of the spine like disc surgery, fixation of spine fractures and correction of spinal deformities.

The Ophthalmology department at Hope Hospital is dedicated to providing state-of-the-art eye care ranging from routine eye examinations to complex surgical procedures. The department also performs refractions, prescribes corrective lenses and performs other tests to help maintain good vision.

Facilities at Hope Hospital include:

- Comprehensive eye care with state of the art equipment
- Glaucoma workup and surgeries
- Other facilities include treatment of retinal disorders and squint surgeries.

The department of Surgery at Hope hospital provides comprehensive round the clock surgical services. The department provides services for all general surgical procedures including advanced gastrointestinal surgeries, rectal surgeries and laparoscopic procedures including advanced laparoscopic surgeries.

The general surgeons work closely with other specialists involved in each patient's care to diagnose and provide timely surgical intervention for complex conditions.

Facilities include:

- Endocrine surgeries
- Gastrointestinal surgeries
- Minimal access surgeries including laparoscopic appendicectomy, gall bladder removal, and hernia repair
- Anorectal surgeries
- Thoracic surgeries
- 24 hrs surgical consultant availability on call basis.

Gynecology:

The department of Gynaecology provides round the clock comprehensive services through a group of qualified and experienced Gynaecologists.

The services offered by the department are as follows:

1. 24 hours emergency service
2. Facilities for gynecological surgeries.
3. Outpatient consultant clinics
4. Inpatient services

Obstetric Procedures are not routinely performed in our organisation unless lifesaving. Caesarean and similar surgeries to save a life of a dying patient may be performed and later transferred to speciality centre specialising in Obstetrics.

The facilities available in the **Department of Otorhinolaryngology (ENT**) provides a range of services through its dedicated , qualified and experienced ENT surgeons.

- Septorhinoplasty
- Nasal Polypectomy
- Tonsil and Adenoid Removal
- FESS
- Oral Surgery
- Oesophagoscopy
- Laryngoscopy and Foreign body Removal
- Mastoid Surgery
- Tympanoplasty
- Gn. Neck Surgery

- Salivary gland surgery
- Audiometry

2. Patient Assessment:

a. Surgical OPD

1. Patient visit the surgical OPD either by themselves anticipating their problem or are guided by the registration counter clerk.
2. Patients are also referred by other clinicians as per the care needed by the patient.
3. Initial Assessment includes patient's past medical history, general examination and routine investigations to confirm patient's need for surgical intervention.
4. Incase the patient need cardiac assessment prior to surgery, the patient is referred to the cardiologist for evaluation.
5. After confirming the patient's need for surgical intervention the plan of care is drawn by the surgeon of the patient.
6. Patients and their relatives are informed about the need for surgery, expected outcome, risk involved, prognosis of the patient etc by the concerned surgeon.
7. The date and time of surgery is scheduled and the patient is informed to get admitted a day before the scheduled date of surgery for undergoing all needed preparation including the required investigations.

b. Emergency Department:

1. Patients directly visit the emergency department of the hospital.
2. The emergency medical officer on duty immediately attends the patients and undertakes the initial assessment to determine the care needed by the patient.
3. Incase the patient need surgical intervention, the surgeon on call is immediately informed by the on duty EMO.
4. The patient is attended by the surgeon who evaluates the patient's condition and the need for surgical intervention.
5. If the patient's need for surgical intervention is not immediate in nature the patient is admitted for further evaluation prior to the surgery.
6. Incase the patient's condition is severe and requires immediate surgical intervention ,the surgeon informs the patient relatives about the need for immediate surgical intervention for saving the life of the patient , outcome expected ,risk involved etc.

3. Preoperative Assessment:

Once the patients need for surgical intervention is established the patients has to undergo a **preoperative checkup by the concerned surgeon** which include the following:

a. Patients Vitals
b. Blood Pressure ,Cardiac Rhythm etc
c. Examination of the chest.
d. Investigation such as X-Ray chest , ECG etc

The patient's preoperative check up findings is **documented** in the specified format in the patients case sheet.

The preoperative check up for patients requiring immediate surgical intervention, is done immediately after the **surgeon's decision** for emergency surgical intervention.

After the preoperative check up by the surgeon, the patient is referred to the anesthetist for preanesthesia check up.

4. Pre anesthesia Check up :

The consultant anaesthetist undertake the preanesthesia check up (ref PA Checkup format) of the patient which include the following :

a. Medical History such as HT/Dm/ Chest Pain etc
b. Surgical and Anesthetics history if any
c. General Examination such as Pulse ,BP,RR etc
d. Systemic Examinations etc

All preanesthesia check up findings are documented in the specified format attached with the patient's bed head ticket (BHT).

The preanesthesia check up for patients requiring immediate surgical intervention ,is done immediately after the surgeon's decision for emergency surgical intervention.

5. Preoperative order to the nursing staff:

A. Ward Nursing Staff:-
The preoperative order to the nursing staff of the concerned ward where the patient is admitted includes the following:

1. Drug orders for example morbidity medications like antihypertensive drugs etc.

2. Patient preparation instructions including marking of the surgical site.etc
3. Time and date of surgery.
4. Specific documents such as X-Ray plates etc, if any to be forwarded along with the patient to the OT.

B. OT Nursing Staff:-

1. List of patient to be operated should be prepared a day prior to the scheduled day of surgery.
2. List should include a patient wise distribution of scheduled surgeries, type of anesthesia to be administered .The information thus prepared is to be informed to the consultant surgeons and anaesthetist.

6. Informed Consent for Surgery:

Consent for the patients and or relatives for the surgery is obtained by the operating surgeon in the specified format after explaining the following details:

1. Nature of Surgery.
2. Reason for the procedure.
3. Expected Outcome.
4. Risk Involved.
5. Expected duration of recovery etc.

The consent is obtained from the patient and or the surrogate (Refer # Informed consent Policy) as per the hospital's policy a day prior to the scheduled date for the surgery.

7. Prevention of Wrong Procedure/Side/Site and Wrong Patient:

The prevention of wrong site/side/procedure and patient begins with the preoperative evaluation of the patient.

This involves the following activities such as marking of the surgical side, verification of patient prior to the shifting of the patient from the ward to the OT, at the time of patient's entrance to the OT suite and prior to the initiation of the procedure inside the theatres ,etc

The policy for prevention of wrong procedure/side/site and patient could be referred for the hospital's detailed policy.

8. Transfer of Patient to Operation Theatre:

a. Prior to the transfer of the patient the ward nurse informs the OT nurse about the patient details and confirms the scheduled time for surgery forty five minutes prior to the scheduled time of surgery.
b. The ward nurse prior to the transfer of the patient ensures that the nurse's preoperative checklist is duly filled and all the patient details as required by the surgeons are arranged in the proper order.
c. The patient is transferred to the OT as per the hospital's intramural transfer policy.

9. Receiving of patient and immediate preoperative assessment:

a. The patient is received in the preoperative patient holding area of the theatre.

b. The OT nurse evaluates the patient's details and checks the preoperative nursing checklist filled by the ward nurse to ensure the patients preparedness and confirms the patient's identity as per the bed head ticket.

c. The patient is transferred to the operating rooms where the concerned surgeon and anesthetist undertakes immediate preoperative evaluation.

10. Post Surgery process:

a. Post Operation the patient is shifted to the post operative recovery area of the OT suite.

b. In the recovery area patient is kept under the supervision of the specialist anesthetist, and consultant surgeon. Nursing care in the postoperative area is provided by Operation Theatre nurses.

c. Prior to discharge from the post operative recovery area **a detailed operative note is documented** by the **concerned surgeon in** the patient's bed head ticket.

d. The consultant surgeon documents the post operative condition of the patient, post operative treatment plan etc. The surgeon concerned will not leave the hospital until and unless he/she is assured about the stable condition of the patient.

e. Post operative treatment plan includes information regarding the need for keeping the patient under intensive care, post operative medications , examinations required if any etc

11. Detailed operative note is documented

Prior to discharge from the post-operative recovery area **a detailed operative note is documented** by the **concerned surgeon in** the patient's IPD file.
If the OT note is documented by a person other than the chief operating surgeon it shall be countersigned by the Chief Surgeon. The detailed operative note should contain information of the procedure performed, postoperative diagnosis, and status of the patient before shifting and orders for post-operative treatment.

12. Adequately spaced, equipped and monitored OT area for infection control practices is available.

In keeping with this, the following must be maintained:

- The operating suite includes operating room, prep/scrub area, clean area, and/or dirty area.
- There is a room dedicated for use as an operating room.
- All surgeries are done in the operating room.
- The operating room is adequately ventilated and temperature controlled.
- There is adequate operating room storage space to hold equipment, sterile supplies and medications. Storage space is adequate to minimize the need to leave the operating room for frequently used supplies, equipment and/or medication.
- There is sufficient storage space that provides easy access for identification and inventory of supplies.
- The operating room is properly cleaned, maintained and free of litter and clutter.

- Each operating room is of a size adequate to allow for the presence of all equipment and personnel necessary for the performance of the surgical procedures, and must comply with applicable local, state or federal requirements. Additionally, OT has triple the minimum of four (4) feet (48 inches) of clear space on each side of the operating table to accommodate emergency personnel and equipment in case of emergency, and permit the safe transfer of the patient to a stretcher for transport
- The emergency criteria, as stated above, can be met in the operating room space available.
- Unauthorized individuals are deterred from entering the operating room suite by facility personnel.
- Sterile supplies are stored away from potential contamination in closed cabinets/drawers or if not, away from heavy traffic areas.
- Sterile supplies are labeled to indicate sterility, and are packaged and sealed to prevent accidental opening.
- Each sterilized pack is marked with the date of sterilization and, when applicable, with the expiration date.
- When more than one autoclave is available, each pack must additionally be labeled to identify in which autoclave it was sterilized.
- Since one sink is used both for dirty instruments and to scrub for surgery, there is a written policy to clean and disinfect the sink prior to scrubbing hands.
- The facility has at least one chemical or gas autoclave.
- Gas sterilizers are vented.
- Where applicable, all instruments used in patient care are sterilized.
- A high-level disinfection is used only for non-autoclavable endoscopic equipment, and in areas that are categorized as semi-critical where contact will be made with mucus membrane or other body surfaces that are not sterile.

- A weekly spore test, or its equivalent, must be performed on each autoclave and the results filed and kept for a minimum of three (3) years.
- If a spore test is positive, there must be a written protocol for remedial action to correct the sterilization process.
- Instrument handling and sterilizing areas must be cleaned and maintained on regularly scheduled basis.
- There must be a strict segregation of dirty surgical equipment and instruments that have been cleaned and are in the preparation and assembly area.
- There is a policy to clean and disinfect the dirty utility area before preparing and assembling packs for sterilization.
- Between operative cases, the operating room(s) must be cleaned with disinfectants according to an established schedule adequate to prevent cross-contamination.
- Scrub suits, caps or hair covers, gloves, operative gowns, masks and eye protection must be utilized for all appropriate surgery.
- A sterile field is routinely used during all operations.
- Surgical scrub soap and/or alcohol cleansers must be provided for the surgery room staff.
- All blood and body fluid spills are cleaned using germicides that are virucidal, bactericidal, tuberculocidal and fungicidal.
- A written protocol has been developed for use by housekeeping personnel for cleaning of floors, tables, walls, ceilings, counters, furniture and fixtures of the surgical suite.
- All openings to outdoor air must be effectively protected against the entrance of insects, animals, etc.
- The operating room ceiling surface or drop-in tiles are smooth, washable and free of particulate matter that can contaminate the operating room.

- The walls and counter tops are covered with smooth and easy to clean material that is free from tears, breaks or cracks.
- The floors are covered with an easily cleaned material which is smooth and free from breaks or cracks. If the floors contain seams or individual tiles, they are sealed with an impermeable sealant other than silicone.
- All equipment is on a preventative maintenance schedule with records kept for a minimum of at least three (3) years. Stickers are placed on individual equipment; however written records must be maintained.
- All equipment repairs and changes are done by a bio-medical technician with records kept for a minimum of three (3) years.
- The operating room must have the following:

✓ An adequate operating room table ,

✓ Adequate lighting in the ceiling,

✓ An EKG monitor with pulse read-out is present,

✓ Blood pressure monitoring equipment is present,

✓ Positive pressure ventilation device (e.g. Ambu® bag),

✓ Source of O2,

✓ Source of suction,

✓ An inspired gas oxygen monitor on the anesthesia machine (if anesthesia is used),

✓ A CO2 monitor is present and is used on all general anesthesia cases (if anesthesia is used), and

✓ An emergency power source, (e.g., a generator), with capacity to operate adequate monitoring, anesthesia (if anesthesia is used), surgical equipment, cautery and lighting for a minimum of two (2) hours.

- ✓ The emergency power equipment is routinely checked to insure proper function, and the test results are filed and kept for a period of three (3) years.
- All medical hazardous wastes are stored in acceptable biohazard containers, and separated from general refuse for special collection and handling.
- Used disposable sharp items are placed in puncture-resistant containers located close to the area in which they are used.
- There is a written policy for cleaning of spills which may contain bloodborne pathogens.

Culture swabs and air sampling plates are sent from Operation Theatres after fumigation every fortnightly.

Monitoring of working OT: Air sampling of a working OT is done once a month. Sampling of in use disinfectants: 1 ml of sample of in-use disinfectants, hand wash agents are sent to microbiology laboratory in a sterile container once a month/ 6 month or annually.

Records are kept with OT in charge. In case of unacceptable results decision on corrective measures are taken by HICC.

13. Quality Assurance Programme for Surgical Services:

Surgical service is a very specialized specialty of medicine and there is a need of quality in everything. Quality in surgical services is achieved through:

- Surgery of any type is done only by appropriately qualified (as per MCI guidelines) and experienced surgeons.
- Any form of Anesthesia is administered by Qualified Anesthetist (MCI guidelines).
- Nursing care is provided by qualified, trained and experienced nursing staff registered with Uttar Pradesh Nursing Council.
- Documentation of patient's surgical plan (Ref Patients Case File)
- Documentation of patient's preoperative condition
  A. Preoperative Surgeons Assessment (Ref Format No #)

B. Preoperative Anesthetist Assessment (Ref Format No #)
C. Preoperative Nursing Checklist. (Ref Format No #)
D. Immediate pre operative assessment of the patient by the consultant surgeon and the anesthetist. (Ref Format No #)

- Transfer of patient to the OT is done by designated staff (Ref Hospital Policy #)
- Time based intraoperative monitoring of the patient by the anesthetist (Ref format #)
- Post surgery monitoring of the patient in the post operative recovery area
- Documentation of the surgical procedures performed, outcome, post operative condition of the patient and post operative treatment plan etc by the consultant surgeon.
- **Appropriate zoning of the operating theatre suite and strict adherence to infection control activities (Ref hospital policy #) in the operating theatre complex.**
- **Well established method for reporting any surgical site infection , its analysis by a multidisciplinary committee for formulation of adequate control and prevention plan**.